



GROWTH & ACQUISITION-READINESS ROADMAP

From *Census Today* to *Value That Sells*

A phased 90-day plan — referrals, community & reputation —
built to grow admissions now and durable enterprise value for sale.

Care That Honors Life

PREPARED FOR

ALEKSANDRA DUBINA · FOUNDER & CEO

CONFIDENTIAL

STRATEGIC WORKING DOCUMENT

Grow census now — build value that transfers

Every move in this roadmap serves two outcomes at once: *more admissions this quarter*, and a business an acquirer will pay a premium for. Hospice buyers underwrite durable, transferable value — stable census, owned referral relationships, clean compliance, and a brand that resists commoditization. Fast census and a sellable agency are not competing goals; the same engine produces both.

ENGINE 1 · NOW

Referral Growth

The fastest lever on census and the single biggest driver of sale multiple. Owned relationships with facilities and physicians are what a buyer is really purchasing.

ENGINE 2 · SOON

Community & Family

Local visibility, reviews, and outreach that capture direct inquiries and reinforce referral credibility — compounding once the referral engine is moving.

ENGINE 3 · COMPOUNDING

Reputation & Press

Founder story, media, and the integrative-care differentiation that make the brand memorable, defensible, and quotable in a diligence room.

FOUNDATION ALREADY IN PLACE

Phase 0 is done. The assets the engine runs on already exist and are on-brand:

✓ Production website & SEO

✓ Referral-partner pitch deck

✓ Die-cut press kit + 4 pillar rack cards

✓ Sound Bath differentiation page

✓ Compliance posture (Medicare · CDPH · ACHC)

✓ The Eternal Standard — four pillars

Three phases, one compounding engine

PHASE 1

Days 1–30

Census Quick Wins

- ∞ Activate the warm network
- ∞ Zero-friction intake SLA
- ∞ Google Profile + reviews live
- ∞ Target list + outreach toolkit
- ∞ First facility field visits

PHASE 2

Days 31–60

Referral Engine

- ∞ Full field cadence (SNF / ALF / MD)
- ∞ Eligibility education & lunch-and-learns
- ∞ Hospital approval started in parallel
- ∞ Community layer begins
- ∞ Track every touch → admit

PHASE 3

Days 61–90

Scale · Community · Press

- ∞ Deepen top-referrer relationships
- ∞ Community events & partnerships
- ∞ Founder story & local press
- ∞ Review velocity engine
- ∞ Month-3 readiness report

The census funnel we are optimizing

Every action below maps to one stage. Move more sources in at the top; remove friction so they convert to admissions faster.

Referral sources activated

Referrals received

Admissions (census in)

Average Daily Census ↑

Leading indicators (sources, touches, referrals) predict the lagging one that buyers price: **stable, growing Average Daily Census.**

Objective: convert relationships and responsiveness into admissions in weeks, not months — before any cold outreach matures.

1

Mine the warm network first

List every past referrer, physician, facility, and personal contact the founder already has. Warm reconnections convert in days; sequence them ahead of cold outreach.

2

Install a zero-friction intake SLA

Same-day eligibility response, 24/7 live answer, next-day admission target. Referrers route patients to whoever admits fastest — this lifts census with the referrals you already get.

3

Turn on local discovery

Optimize the Google Business Profile and launch a steady review request to families served. Captures “hospice near me” inquiries and builds referral-room credibility.

4

Deploy the target list + outreach toolkit

Prioritized facilities and physician groups across Ventura & LA, plus intro scripts, an eligibility one-pager, and the rack-card leave-behind — ready to work day one.

5

First field visits — lowest-gatekeeper sources

Board-and-care homes, assisted living, and skilled nursing first: eligible patients now, fewer approval barriers than hospitals. Earliest admissions live here.

PHASE 1 KPIS

Warm contacts re-engaged

Referral sources visited

Time-to-admit (hours)

First new admissions

Google reviews added

Objective: turn early wins into a repeatable, tracked cadence that compounds — and that transfers to a buyer as an owned asset, not a founder's address book.

1

Run a weekly field cadence

Standing visit rhythm across SNFs, ALFs, physician practices (oncology, cardiology, pulmonology, nephrology/dialysis, geriatrics), and home-health step-down partners.

2

Lead with eligibility education

Most patients are referred *too late*. Disease-specific guidelines and the "surprise question" help partners refer earlier — directly increasing census and length of stay.

3

Start hospital approvals in parallel

Begin vendor-credentialing and discharge-planner relationships now. It is a slow burn — never gate early census on it, but it unlocks volume in the following quarter.

4

Track every touch in one place

A simple referral tracker logs accounts, contacts, touches, and referrals→admissions. This is both a management tool and the evidence of a transferable pipeline at diligence.

5

Begin the community layer

Senior centers, faith communities, and ALF family talks — seeding direct inquiries and reinforcing the brand the referral engine depends on.

PHASE 2 KPIS

Active referral sources

Referrals / week

Referral → admit conversion %

Admissions / week

Average Daily Census trend

Hospital approvals in progress

Objective: deepen what works, add visibility that compounds, and produce the evidence that supports the equity decision at the end of the test phase.

1

Double down on top referrers

Identify the sources driving the most admissions and invest in standing relationships, responsiveness, and service recovery. A few deep accounts beat many shallow ones.

2

Community awareness events

Lunch-and-learns, senior-org partnerships, and caregiver education. Positions Eternal as the local authority and feeds both direct inquiries and referral trust.

3

Press & the founder story

Local media, accreditation milestones, and the integrative-care (Sound Bath) differentiation — a genuinely newsworthy, compliant angle that competitors cannot easily copy.

4

Build review velocity

Systematic, compliant review requests at the right moments. Volume and recency of reviews lift both family discovery and referrer confidence.

5

Deliver the month-3 readiness report

A clean dashboard of referrals, admissions, ADC trend, and the now-documented engine — the basis for the equity conversation and an early diligence artifact.

PHASE 3 KPIS

ADC growth vs baseline

Top-referrer concentration

Community / press placements

Review count & rating

Documented, transferable pipeline

What turns growth into a premium sale

The same engine that grows census builds the four things hospice acquirers actually pay multiples for. Every phase above is engineered to strengthen these.

Stable, growing census

Predictable Average Daily Census is the headline number in any valuation. Consistency and trend matter as much as size.

Owned referral relationships

Documented, multi-source, not founder-dependent. A diversified, transferable pipeline de-risks the deal for a buyer.

Clean compliance

Medicare · CDPH · ACHC in good standing, with Anti-Kickback / Stark-safe practices. Diligence risk is what kills hospice deals.

Defensible brand & differentiation

The Eternal Standard four pillars + integrative modalities make the agency memorable and harder to commoditize.

THE SCOREBOARD

METRIC	TYPE	WHY IT MATTERS
Average Daily Census (ADC)	Lagging	The number buyers price the business on.
Admissions / week	Lagging	Direct census growth; the pulse of the engine.
Active referral sources	Leading	Pipeline breadth and transferability.
Referral → admit conversion	Leading	Intake efficiency; where census quietly leaks.
Time-to-admit	Leading	Responsiveness — why referrers choose you.
Median length of stay	Lagging	Earlier referrals via eligibility education.
Reviews — count & rating	Leading	Family discovery and referrer confidence.

COMPLIANCE GUARDRAIL — NON-NEGOTIABLE

All referral development is built on **value and quality of care** — clinical responsiveness, education, and service — never inducements. Every tactic here stays within Anti-Kickback Statute and Stark boundaries. Spotless compliance is not a constraint on the sale; it is one of the assets being sold.

THE 90-DAY CHECKPOINT

Grow census now. *Build the asset* as we go.

Phase by phase, this plan turns warm relationships and fast intake into admissions today — and into the durable, transferable value that defines the equity conversation at the end of the test phase.

INTAKE / REFERRALS

805.953.7273

EMAIL

info@eternallifehospice.com

ONLINE

eternallifehospice.com

MEDICARE-CERTIFIED · CDPH-LICENSED · ACHC-ACCREDITED · VENTURA & LOS ANGELES COUNTY